

SHRI DANESHWARI DIAGNOSTIC LABORATORY

vidya ganapati temple,anekal main road jigani-566105 -mob -9945624698.



COMPLETE SOLUTION FOR ALL TYPES OF PATHOLOGICAL INVESTIGATION DONE ON FULLY AUTOMATED ANALYZER.

Patient Name : **MR. VENKATESH**
Age & Sex : 54 Yrs | Male
Address :
Mobile No. :
Referred By : **Dr PARASHURAM**

Lab ID. No. : 3521
Collection Date : 13-05-2025 19:21
Reporting Date : 13-05-2025 19:54
Collection Centre : -



Test Name	Results	Units	Reference range
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KIDNEY FUNCTION PARAMETERS

Creatinine

0.6

mg/dL

0.6-1.2

Method : Automated Spectrophotometry Based Assay.

Creatinine is a waste product that comes from the normal wear and tear on muscles of the body. Everyone has creatinine in their bloodstream. The kidneys maintain the blood creatinine in a normal range. Creatinine has been found to be a fairly reliable indicator of kidney function with elevated level may signifies impaired kidney function or kidney disease.

— End of report —

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Collection Date : **13-05-2025 19:21**

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Reporting Date : **13-05-2025 19:31**

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BIOCHEMISTRY

Nephelometric (Quantitative)

C Reactive Protein (CRP)

▲ **24.9** mg/L

< 6.0

Method : Automated Nephelometric immunoassay based assay.

CRP is an acute phase reactant which is used in inflammatory disorders for monitoring course and effect of therapy. It is most useful as an indicator of activity in rheumatoid arthritis, Rheumatic fever, tissue injury or necrosis and infections. As compared to ESR, CRP shows an earlier rise in inflammatory disorders which begins in 4-6 hrs, the intensity of the rise being higher than ESR and the recovery being earlier than ESR. Unlike ESR, CRP levels are not influenced by hematologic conditions like Anemia, Polycythemia etc.



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Test Name	Results	Units	Reference range
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HEMATOLOGY

COMPLETE BLOOD COUNT (CBC)

Hemoglobin	14.1	g/dL	13.5-17.5
Total Leukocyte Count (TLC)	▲ 13400	/cumm	4,500-11,000
Differential Leucocyte Count (DLC)			
Neutrophils	▲ 90	%	35-66
Lymphocytes	▼ 05	%	24-44
Monocytes	▼ 02	%	4-10
Eosinophils	03	%	0-3
Red Blood Count (RBC)	5.16	million/ μ l	4.5 - 5.9
Packed Cell Volume (PCV)	41.5	%	37-53
Mean Corpuscular Volume (MCV)	80.4	fL	80 - 100
Mean Corpuscular Hemoglobin (MCH)	27.3	pg	26 - 34
Mean Corpuscular Hb Concentration (MCHC)	34.0	g/dL	32 - 36
RDW-SD	44.8	fL	37 - 54
RDW-CV	▲ 14.8	%	11.6 - 14

Absolute Leucocyte Count

Absolute Neutrophils Count	▲ 12.1	thou/mm ³	2.00 - 7.00
Absolute Lymphocytes Count	▼ 0.7	thou/mm ³	1.00 - 3.00
Absolute Monocytes Count	0.3	thou/mm ³	0.20 - 1.00
Absolute Eosinophils Count	0.4	thou/mm ³	0.02 - 0.50
Platelet Count	316000	thou/ μ l	150000 - 450000
Mean Platelet Volume	10.4	fL	6.5 - 12.0
Platelet Haematocrit (PCT)	▲ 0.33	%	0.1 - 0.28
Platelet Distribution Width (PDW)	▼ 12.0	fL	15 - 18

PERIPHERAL BLOOD SMEAR





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LFT(LIVER FUNCTION TEST)

SERUM BILIRUBIN(TOTAL)

Method: DIAZO METHOD

▲ 5.9 mg/dL 0.2 - 1.2

SERUM BILIRUBIN(DIRECT)

Method: DIAZO METHOD

▲ 4.2 mg/dL 0.0 - 0.2

SERUM BILIRUBIN(INDIRECT)

Method: Calculated

▲ 1.7 mg/dL 0 - 0.8

Aspartate Transaminase (AST/SGOT)

Method: IFCC METHOD

▲ 225 U/L 0 - 50

Aspartate Transaminase (ALT/SGPT)

Method: IFCC METHOD

▲ 212 U/L 0 - 45

Alkaline Phosphatase

Method: TRIS CARBONATE BUFFER, KINETIC

▲ 635 mg/dL 60-306

Total Protein

Method: BIURET

7.9 g/dL 6.4-8.2

Albumin

Method: BROMOCRESOL GREEN

▼ 2.4 g/dL 3.4-5

Globulin

Method: BROMOCRESOL GREEN

▲ 5.5 g/dL 1.9-3.9

A/G ratio

Method: KINETIC

▼ 0.44 - 1.0 - 2.0

Method : Automated Spectrophotometry based Assay.

Note

1. In an asymptomatic patient, Non alcoholic fatty liver disease (NAFLD) is the most common cause of increased AST, ALT levels. NAFLD is considered as hepatic manifestation of metabolic syndrome.
2. In most type of liver disease, ALT activity is higher than that of AST; exception may be seen in Alcoholic Hepatitis. Hepatic Cirrhosis, and Liver neoplasia. In a patient with Chronic liver disease, AST:ALT ratio >1 is highly suggestive of advanced liver fibrosis.
3. In known cases of Chronic Liver disease due to Viral Hepatitis B & C, Alcoholic liver disease or NAFLD, Enhanced liver fibrosis (ELF) test may be used to evaluate liver fibrosis.
4. In a patient with Chronic Liver disease, AFP and Des-gamma carboxyprothrombin (DCP)/PIVKA II can be used to assess risk for development of Hepatocellular Carcinoma.

